IN THE DISTRICT COURT OF APPEAL OF FLORIDA
THIRD DISTRICT

Case No.: 3D26–
L.T. Case No.: 2026012883

DAISY FUNDORA & ASSOCIATES, INC., d/b/a OUR DREAM ALF,
Petitioner,

v.

FLORIDA AGENCY FOR HEALTH CARE ADMINISTRATION,
Respondent.

Original Proceedings for Review of a Nonfinal Order of the Agency for Health Care Administration

PETITION FOR REVIEW OF EMERGENCY SUSPENSION ORDER

Dwight O. Slater, B.C.S.
Florida Bar No.: 0030607
dwight@guildaylaw.com
GUILDAY LAW, P.A.
1566 Village Square Boulevard
Tallahassee, Florida 32309
T: 850.224.7091
F: 850.222.2593
Counsel for Petitioner

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BASIS FOR INVOKING THE COURT'S JURISDICTION

Petitioner seeks review of an Emergency Suspension Order issued by the Agency for Health Care Administration in accordance with sections 120.60(6) and 120.68, as well as Florida Rules of Appellate Procedure 9.100 and 9.190(b)(2).

FACTS UPON WHICH PETITIONER RELIES

Daisy Fundora & Associates, Inc., d/b/a Our Dream ALF ("Our Dream" or the "facility") operates an assisted living facility in Miami-Dade County. [Appx. 2–3]. For many of its residents, the facility has been their home for years. Some have lived there for decades. Over that time, the residents and the family that owns and operates Our Dream have developed longstanding personal relationships; residents have watched members of the owners' family grow up, and the owners regard the residents as members of their extended family. [Appx. 37–38].

AHCA commenced a re-licensure survey of the facility on July 20, 2026, and returned on July 22, 25, and 27. On July 28, AHCA issued the Emergency Suspension Order ("ESO") at issue here. [Appx. 15]. The Order did not require immediate closure. Instead, AHCA made the suspension effective three days later, on July 31, 2026, at 5:00 p.m. [Appx. 2–35].

The ESO identifies various alleged deficiencies across several aspects of the facility's operation. Much of the Order concerns medication management. AHCA alleges deficiencies in Medication Observation Records ("MORs"), including incomplete or inaccurate records and failures to document assistance with medications contemporaneously. AHCA also

alleges that medications associated with numerous residents were found unused in the bathroom of the administrator's office, although MORs reflected that some of those medications had been provided. AHCA also alleges that surveyors observed medications for twenty-nine residents pre-poured into cups before a morning medication pass, while MORs had already been signed to indicate that medications had been provided, and that a staff member assisting residents could not identify which medications were intended for which residents. [Appx. 20–22]. From these circumstances, AHCA alleges that residents did not receive medications as prescribed. [Appx. 16–18, 29].

The ESO also alleges deficiencies in communicating with health-care providers and reconciling medications following hospitalization or other changes in a resident's condition. As to one resident, AHCA alleges that Metformin prescribed in April 2026 was not documented as filled or administered during April and May and that staff failed to confirm the medication with the resident's health-care provider. AHCA alleges that this placed the resident "at risk of dangerously high blood glucose levels." [Appx. 16–18, 29].

AHCA separately alleges deficiencies concerning therapeutic diets. According to the ESO, seven residents prescribed diabetic diets did not receive diets conforming to those prescriptions, one resident did not receive a prescribed soft diet, and the facility failed adequately to account for a resident's peanut allergy. AHCA alleges that these deficiencies placed residents "at risk" of allergic reactions, unsafe blood-glucose levels, injury, or death. [Appx. 23–24, 29–30].

The Order also identifies physical-plant and emergency-preparedness deficiencies. AHCA alleges that the facility's two portable generators were not operational during its survey visits and that the facility did not maintain the required quantity of emergency fuel. [Appx. 25, 29]. It alleges that a deadbolt on the front door required a key to unlock from the inside, such that staff would need to locate and use the key in an emergency. [Appx. 25–26]. And AHCA alleges that indoor temperatures measured between 82 and 85 degrees during one of its visits. [Appx. 26, 29].

AHCA further alleges that the facility was inadequately staffed. According to the ESO, two staff members were providing care to approximately twenty-nine residents, many of whom had mental-health conditions or other needs. The Order also alleges deficiencies involving resident supervision and follow-up care, including failures to notify health-care providers of significant changes in residents' conditions, medication refusals, hospitalizations, and falls. As to one resident who was later hospitalized following an episode of altered mental status, AHCA alleges that the facility lacked documentation of follow-up care and interventions following earlier falls. AHCA also alleges that, when another resident was found unresponsive and later died at the hospital, staff failed properly to perform CPR, use the facility's AED, or timely contact 911. [Appx. 16–20, 24–26]. These incidents were not confined to the July survey period; the ESO points to falls, hospitalizations, medication issues, and alleged failures of provider notification dating from January through June 2026. [Appx. 16–20, 24–25].

AHCA does not rely on any one of these allegations alone. Rather, the ESO characterizes the alleged deficiencies "individually and collectively" as demonstrating "systemic" management failures and an inability or unwillingness to comply with the regulatory scheme. AHCA concludes that the deficiencies create a "current and ongoing threat" to the facility's residents and potential residents. [Appx. 30–32].

Based on that collective assessment, AHCA ordered the emergency suspension of Our Dream's license and required the facility to cease operating effective July 31 at 5:00 p.m. [Appx. 34–35]. The suspension thus required the residents then living at Our Dream to be placed elsewhere. [Appx. 35].

This petition followed.

NATURE OF THE RELIEF SOUGHT

Petitioner seeks an order quashing and vacating the Emergency Suspension Order.

ARGUMENT

I. The ESO fails to allege particularized facts demonstrating an immediate serious danger and, even if it did, complete suspension is not narrowly tailored to the danger alleged.

An emergency suspension order under section 120.60(6), Florida Statutes, is reviewed by petition under Florida Rule of Appellate Procedure 9.190(b)(2). Review is confined to the four corners of the order, with the agency's factual allegations accepted as true. The question is whether those allegations are facially sufficient to justify emergency action.

Generally, the order must allege particularized facts demonstrating an "immediate serious danger to the public health, safety, or welfare." St. Michael's Acad., Inc. v. Dep't of Children & Families, 965 So. 2d 169, 171–73 (Fla. 3d DCA 2007). Specifically, the facts alleged must establish that (1) the complained-of conduct is likely to continue; (2) emergency action is necessary to stop the danger; and (3) the action taken is sufficiently narrowly tailored to be fair. Bio-Med Plus, Inc. v. Dep't of Health, 915 So. 2d 669, 672 (Fla. 1st DCA 2005). Every fact necessary to satisfy those requirements must appear on the face of the order; conclusory predictions of future harm and allegations establishing regulatory violations alone are insufficient. St. Michael's, 965 So. 2d at 171–73; Kaplan v. Dep't of Health, 45 So. 3d 19, 20 (Fla. 1st DCA 2010). And even when an immediate serious danger is adequately alleged, section 120.60(6) permits the agency to take "only that action necessary to protect the public interest," requiring a remedy narrowly tailored to the danger identified.

A. The ESO fails to allege particularized facts establishing an immediate serious danger.

AHCA's ESO is detailed, but its detail is directed principally toward establishing regulatory violations. The Order identifies statutes and rules governing assisted living facilities, alleges conduct violating those provisions, and catalogs deficiencies across various aspects of Our Dream's operation. Those allegations may support administrative charges. But section 120.60(6) requires more for emergency action.

Florida courts repeatedly emphasize that "it is not enough for the [ESO] merely to allege statutory violations." Bio-Med Plus, Inc., 915 So. 2d

at 672. The inquiry focuses "not simply on charges of statutory violations," but on the "particularized facts which demonstrate an immediate danger to the public." Kaplan, 45 So. 3d at 20. And conduct warranting discipline through the ordinary administrative process does not necessarily establish the immediate danger required for emergency action. See Daube v. Dep't of Health, 897 So. 2d 493, 495 (Fla. 1st DCA 2005).

Here, even accepting the ESO's allegations as true, AHCA fails to meet its burden. The Order moves from the facts that potentially establish a violation to the conclusion of an immediate serious danger without alleging the particularized facts necessary to connect the two.

1. The medication allegations do not allege particularized facts establishing a present and continuing serious danger.

The largest category of allegations concerns medication management. AHCA alleges inaccurate or incomplete Medication Observation Records ("MORs"); failures to document medication assistance contemporaneously; handwritten MORs; unused medications found in the administrator's office bathroom; failures to reconcile medications following hospitalization; failures to contact health-care providers regarding medication refusals or changes in condition; and instances in which AHCA concludes that prescribed medications were not provided.

Even accepting those allegations as true, at most they allege deficiencies in medication management and recordkeeping. They lack the necessary particularized facts to demonstrate an immediate serious danger. The unused medication allegations illustrate the problem. AHCA alleges that medications associated with residents remained unused while MORs

reflected that medications had been administered. That discrepancy may support AHCA's claim that the records were inaccurate and may even support an inference that some medications were not administered as prescribed. But the ESO does not allege as to many of those medications which medically necessary dose a particular resident failed to receive, when it should have been administered, whether the resident was present at the facility when it was due, whether the medication had been refused, held, changed, or discontinued, what medical consequence resulted from its omission, or why the omission presented a continuing danger. Those are not factual disputes with the Order; they are required elements the Order does not allege.

Instead, the ESO relies on a chain of inferences: unused medication → prescribed medication was not administered → medically necessary treatment was withheld → the omission created a serious medical danger → that danger remained present and continuing when the ESO issued. The first allegation does not, standing alone, establish every proposition that follows. Section 120.60(6) requires the particularized facts necessary to establish the emergency itself.

Resident #1 provides a concrete example. AHCA alleges that Metformin was prescribed in April, that the facility could not establish that it was filled or administered during April and May, and that blood glucose above a specified level can be dangerously high. From those allegations, AHCA concludes that the resident was placed "at risk of dangerously high blood glucose levels." [Appx. 16–18, 29]. This claim is insufficient on its face. The Order alleges a medication-management violation and a "risk" of a

potentially serious medical consequence. It does not allege that Resident #1 experienced dangerously elevated blood glucose because Metformin was not administered, that the medication remained prescribed and medically necessary when the ESO issued in late July, or that the alleged failure to provide it was then continuing.

The same defect appears in AHCA's allegations concerning medication reconciliation and provider communications. Failure to maintain an accurate MOR, reconcile medications after hospitalization, or contact a provider when required may establish a regulatory violation. But the ESO must still allege the particularized facts demonstrating how the identified violation created a present and continuing serious danger. The regulatory violation itself does not supply that missing element.

2. The dietary allegations identify potential risks without alleging facts establishing their immediacy.

AHCA alleges that seven residents did not receive prescribed diabetic diets, one resident did not receive a prescribed soft diet, and the facility failed adequately to account for a resident's peanut allergy. Again, at most those allegations establish possible violations of the rules governing resident care; absent are facts demonstrating that those violations created an immediate serious danger.

The ESO does not allege that any diabetic resident experienced dangerously elevated glucose because of the food served; that the resident prescribed a soft diet choked, aspirated, or was otherwise in immediate danger because the diet was not followed; or that the resident with the

peanut allergy was exposed to peanuts or faced an identified imminent exposure.

Instead, AHCA alleges that these deficiencies placed residents "at risk" of allergic reactions, unsafe glucose levels, injury, or death. "At risk of harm" describes a possibility; "immediate serious danger" describes a present threat. The former does not, without particularized facts establishing imminence and seriousness, establish the latter.

3. The physical-plant and emergency-preparedness allegations likewise fail to allege an immediate serious danger.

AHCA alleges that the facility's portable generators were not operational during its survey visits and that the facility lacked the required emergency fuel. Again, these allegations at best establish an emergency-preparedness deficiency. But the danger associated with an emergency generator depends upon another event: interruption of primary electrical power. The ESO does not allege that primary power had failed, that a hurricane or other event threatening electrical service was imminent, or that any other circumstance made a loss of power an immediate contingency when the ESO issued. In other words, the ESO alleges inadequate preparation for a possible emergency—far short of an immediate serious danger.

The keyed-deadbolt allegation presents the same problem. AHCA alleges that one of the facility's doors required a key for egress and that staff would have to locate and use the key in an emergency. This establishes an egress deficiency. There is no claim that residents were unable to exit the facility, that the door was their only safe means of egress, that the necessary

key was unavailable, that anyone had previously been unable to leave, or that an evacuation was occurring or imminent. Again, these allegations describe a situation that could become dangerous upon the occurrence of another event. It does not allege the particularized facts establishing that such an event was imminent.

The temperature allegation involves a condition that existed when observed. AHCA alleges indoor temperatures between 82 and 85 degrees, above the regulatory maximum. But the ESO does not allege that any resident suffered heat-related illness or distress, how long the elevated temperatures persisted, or resident-specific facts establishing that the observed temperatures created an immediate serious danger. In short, these allegations may constitute violations of safety regulations, but they do not amount to a statutory emergency authorizing immediate suspension.

4. The staffing, documentation, and operational allegations do not allege the missing emergency either.

AHCA also alleges insufficient staffing, failures to maintain required records, failures to notify health-care providers, deficiencies in incident reporting, and other operational violations. These are quintessential regulatory allegations. Missing documentation, an untimely report, or failure to maintain a required record may establish noncompliance; but the ESO must still demonstrate how that noncompliance created an immediate serious danger to residents. Here, it fails yet again.

AHCA alleges that two staff members were caring for approximately twenty-nine residents, many of whom had mental-health conditions or other needs. But the Order does not allege what staffing level was required and

absent, what particular resident needs could not be met because only two employees were present, or what immediate serious danger resulted from that staffing level.

The same applies to the allegations concerning provider notification and resident records. A failure to document provider contact does not itself establish that medically necessary communication did not occur; and even an alleged failure to make required contact must still be connected by particularized facts to a present and continuing danger when emergency action was taken.

Across these categories, the defect is the same: AHCA alleges regulatory violations and leaps from violation to emergency. Section 120.60(6) requires the particularized facts that bridge that gap.

B. Calling disparate deficiencies "systemic" does not transform them into an immediate danger.

AHCA attempts to supply the missing nexus through aggregation. After cataloguing deficiencies involving medication management, provider communications, therapeutic diets, staffing, temperature, emergency power, egress, and other aspects of facility operations, the ESO characterizes them as "collective failures" and "systemic management failures," concludes that Our Dream is "unable or unwilling" to comply with the regulatory scheme, and declares the result a "current and ongoing threat" to residents.

But those are AHCA's characterizations; they are not facts. And Florida cases distinguish between an agency's conclusion that multiple deficiencies collectively demonstrate an emergency and particularized facts

showing that the deficiencies actually converge upon a single, present danger.

This Court's decision in St. Michael's illustrates the former. 965 So. 2d 169 (Fla. 3d DCA 2007). There, DCF relied on several different deficiencies—including a child wandering from the facility, personnel-record deficiencies, a child-biting incident, and a licensing violation—and combined them with the facility's "poor compliance history" to support emergency suspension. Id. at 171. This Court examined the facts underlying the individual allegations and was "not persuaded by conclusory predictions of future harm based on factual allegations which do not demonstrate an immediate danger." Id. at 173.

The First District reached a similar result in Premier Travel International, Inc. v. Department of Agriculture & Consumer Services, 849 So. 2d 1132 (Fla. 1st DCA 2003). Although the agency characterized numerous incidents as a "systematic course of conduct," the court found that characterization was itself conclusory and held that the order still had to allege facts establishing the requisite continuing threat. Id. at 1136.

The cases sustaining emergency action based on multiple deficiencies demonstrate what was missing in St. Michael's, Premier Travel, and what is missing here: a factual nexus connecting the deficiencies to a single, demonstrated, ongoing danger. In Rehabilitation Center at Hollywood Hills, LLC v. Agency for Health Care Administration, 250 So. 3d 737 (Fla. 1st DCA 2018), the agency did not merely aggregate unrelated violations and label them systemic. During an actual, unfolding heat emergency, residents suffered serious medical distress one after another,

staff repeatedly failed to recognize and respond appropriately, and eight residents died. The different failures thus demonstrated a single, present institutional incapacity—the facility's inability to recognize and respond safely to the very emergency then occurring.

Additionally, Cameron v. Department of Health, 301 So. 3d 503 (Fla. 1st DCA 2020), and Daniels v. Department of Health, 301 So. 3d 503 (Fla. 1st DCA 2020), demonstrate a similar pattern. There, multiple COVID-control deficiencies were connected by an alleged factual consequence: the failures had "resulted in the spread of COVID-19 in the nursing home facility." Id. at 504. The allegations therefore did not merely identify several different risks. They tied those deficiencies to a single, actual, ongoing harm.

Unlike in Hollywood Hills, where multiple failures demonstrated one institutional incapacity during an actual emergency that had already resulted in eight deaths, or in Cameron and Daniels, where multiple deficiencies were factually connected to an actual outbreak within the facility, the ESO in this case provides no such nexus. Indeed, the ESO here is more in the vein of St. Michael's, where the agency assembled different violations and a poor compliance history and predicted that their cumulative effect presented an immediate danger. This Court quashed the order in that case and should do so here as well.

The ESO alleges no common event or present harm connecting its disparate deficiencies. The medication-record deficiencies do not cause the generator deficiency. The generator deficiency does not cause the therapeutic-diet violations. The dietary allegations do not cause the keyed-deadbolt condition. The deadbolt does not cause deficient provider

communications. And the temperature reading does not explain the unused medications.

AHCA's asserted nexus is instead the number and variety of the violations themselves. From them, it infers that management is systemically incapable of operating the facility safely and that the individual risks therefore amount collectively to an immediate serious danger. But that is the very inferential step for which the ESO must supply particularized facts. Calling the deficiencies "systemic," "collective," or evidence that Our Dream is "unable or unwilling" to comply does not supply them.

Nor does AHCA's aggregation of historical events establish the required immediacy. In finding an emergency on July 28, the ESO combines conditions observed during the July survey with falls, hospitalizations, medication issues, and alleged notification deficiencies occurring over the preceding several months. [Appx. 16–20, 24–25, 28–32.] Those historical events may bear on regulatory compliance, but their accumulation does not establish that an "immediate serious danger" existed on July 28. An agency cannot transform a series of past deficiencies into a present emergency merely by aggregating them; the ESO must allege particularized facts demonstrating a danger existing at the time emergency action was taken.

In sum, the allegations in the ESO may support an administrative complaint, corrective action, penalties, or ultimately discipline if proven. But a detailed administrative complaint does not become an emergency suspension order merely by adding the words "systemic," "collectively," and "ongoing threat." Section 120.60(6) required particularized facts demonstrating the emergency itself. The ESO in this case has none.

C. Even if some allegations established an immediate danger, complete suspension was neither necessary nor narrowly tailored to that danger.

Even if one or more allegations in the ESO sufficiently established an immediate serious danger, that does not end the inquiry. Section 120.60(6) separately limits an agency to "only that action necessary to protect the public interest under the emergency procedure." Bio-Med Plus, Inc., 915 So. 2d at 672. Thus, an emergency order must establish not only a danger warranting immediate action, but also that the particular action imposed is necessary and sufficiently tailored to that danger. Id.

Florida courts enforce that limitation independently. In Nath v. Department of Health, 100 So. 3d 1273, 1276 (Fla. 1st DCA 2012), for example, the First District did not question the need for immediate action but quashed the emergency suspension because the order failed to explain why less restrictive measures would not adequately protect the public. Likewise, Cameron and Daniels involved allegations sufficient to establish an immediate danger from the spread of COVID-19 in a nursing facility. Yet the court still quashed the emergency suspensions because the orders did not sufficiently explain why complete suspension, rather than less restrictive measures, was necessary.

That distinction is particularly important here. The dangers alleged in the ESO correspond almost one-for-one with targeted measures capable of addressing them. If the generators created an immediate danger, AHCA could require operational generators and the required fuel. If the keyed deadbolt created an immediate egress danger, AHCA could require correction of the lock. If excessive indoor temperature

endangered residents, AHCA could require restoration and monitoring of compliant temperatures. If two staff members were insufficient to meet resident needs, AHCA could require additional staffing. If medication-management practices created an immediate danger, AHCA could require medication reconciliation, current MORs, qualified medication personnel, additional supervision, or professional oversight. If prescribed diets or allergy precautions were not being followed, AHCA could require immediate compliance and verification.

These are not hypothetical alternatives unrelated to AHCA's concerns. Each is directed at eliminating the very condition the ESO identifies as dangerous. And because the alleged deficiencies are different, the appropriate corrective measures are correspondingly different. Nothing inherent in an inoperable generator, a keyed lock, an elevated temperature, inadequate staffing, deficient medication practices, or noncompliant diets explains why the only means of protecting residents from those conditions was to close the facility.

AHCA also possessed an intermediate remedy directed specifically at continued operation: it could restrict new admissions while requiring correction of conditions affecting existing residents. See § 408.814, Fla. Stat. The availability of such measures does not mean AHCA was required to select any particular one. But it underscores what the ESO itself was required to explain: why measures directed at the alleged dangers would not adequately protect the residents already living there.

The ESO provides no such explanation. Instead, AHCA simply states that it considered less restrictive alternatives but determined that they would

not adequately protect residents because the deficiencies were systemic and demonstrated that Our Dream was unable or unwilling to comply. That merely restates AHCA's conclusion. It does not explain why correction of the deficiencies or something short of complete suspension would not suffice.

Again, Hollywood Hills is instructive. There, the alleged danger was not a collection of discrete conditions that could each be eliminated by correcting the condition itself. The allegations demonstrated a facility-wide inability to recognize and respond to an actual medical emergency—an institutional failure that had already manifested itself in multiple deaths. On those allegations, the asserted danger inhered in the facility's continued operation itself.

AHCA's allegations here are materially different. The ESO identifies conditions and practices that can be corrected, then infers from their number that the facility itself must be closed. But the narrow-tailoring requirement prevents the Agency from making that leap without explaining why correction of the alleged dangers would be inadequate.

It is worth noting that AHCA's own chronology further underscores the problem. The Agency began its survey on July 20, returned on July 22, 25, and 27, and alleges that several conditions it ultimately characterized as dangerous persisted throughout that period. AHCA nevertheless permitted residents to remain in the facility. Even after issuing the ESO on July 28, it delayed the suspension until July 31 at 5:00 p.m. That chronology does not foreclose emergency action, but it is difficult to reconcile with the proposition that immediate removal of every resident was the only measure capable of protecting them.

Complete suspension carried consequences that narrower measures would not. The residents were not simply customers of a regulated business. For many, Our Dream was their home. Complete suspension required them to leave familiar surroundings, caregivers, routines, and relationships. Section 120.60(6)'s command that an agency take "only that action necessary" has particular force where the remedy selected to protect vulnerable residents itself requires their displacement.

The ESO does not explain why correction of the alleged violations was insufficient. It identifies discrete dangers, each susceptible to a targeted response, and then relies on its characterization of the violations as "systemic" to justify the broadest available remedy. Section 120.60(6) requires more. Even assuming emergency action was warranted, AHCA was required to allege particularized facts demonstrating why complete suspension—and the resulting displacement of every resident—was necessary to eliminate the danger. It failed to do so.

CONCLUSION

For the foregoing reasons, Our Dream respectfully asks this Court to grant its petition and quash the ESO.

Respectfully submitted,

/s/ Dwight O. Slater
Dwight O. Slater, B.C.S.
Florida Bar No.: 0030607
dwight@guildaylaw.com
GUILDAY LAW, P.A.
1566 Village Square Boulevard
Tallahassee, Florida 32309
T: 850.224.7091
F: 850.222.2593
Secondary
email: stephanie@guildaylaw.com
Counsel for Petitioner

CERTIFICATE OF COMPLIANCE

I certify that this petition complies with the font and word count requirements of Florida Rules of Appellate Procedure 9.045 and 9.210. This petition is computer-generated, is set in Arial 14-point font, and contains 4245 words, excluding the parts of the document exempted by Rule 9.045(e).

/s/ Dwight O. Slater
Dwight O. Slater, B.C.S.
Florida Bar No.: 0030607
dwight@guildaylaw.com
GUILDAY LAW, P.A.
1566 Village Square Boulevard
Tallahassee, Florida 32309
T: 850.224.7091
F: 850.222.2593
Secondary
email: stephanie@guildaylaw.com
Counsel for Petitioner

CERTIFICATE OF SERVICE

I CERTIFY that on this 25th day of August, 2026, a true and correct copy of the foregoing has been served by e-mail via the Florida Courts e-Filing Portal on Glenn Rininger, General Counsel, Office of the General Counsel, Agency for Health Care Administration, 2727 Mahan Dr., M.S. #3, Tallahassee, FL 32308, at glenn.rininger@ahca.myflorida.com, and Thomas M. Hoeler, Chief Facilities Counsel, Office of the General Counsel, Agency for Health Care Administration, 2727 Mahan Dr., M.S. #3, Tallahassee, FL 32308, at thomas.hoeler@ahca.myflorida.com.

/s/ Dwight O. Slater
Dwight O. Slater, B.C.S.
Florida Bar No.: 0030607